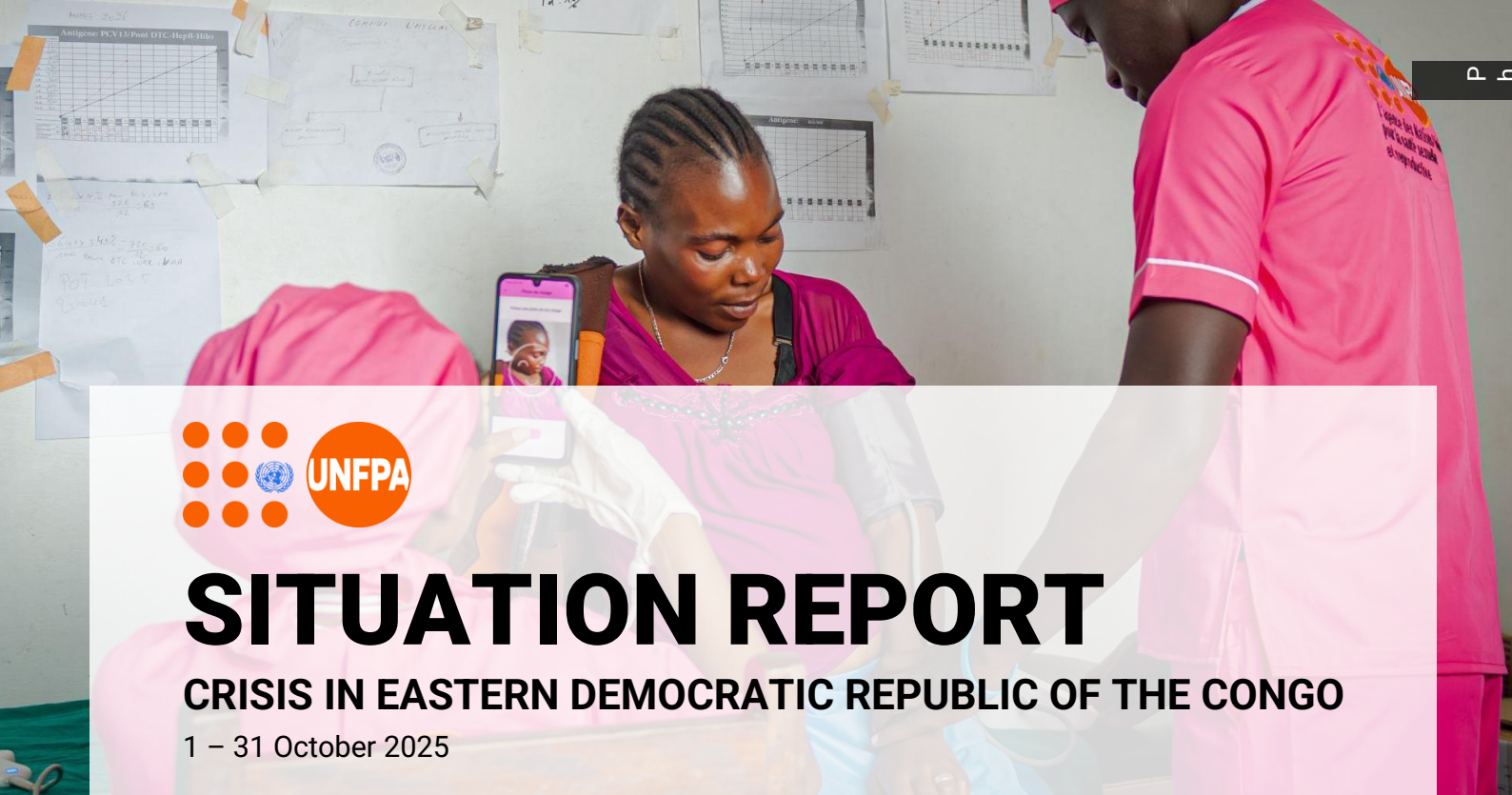




SITUATION REPORT

CRISIS IN EASTERN DEMOCRATIC REPUBLIC OF THE CONGO

1 – 31 October 2025

Highlights

In October 2025, the humanitarian crisis in eastern Democratic Republic of the Congo (DRC) deteriorated rapidly due to intensified armed conflict, causing a 15.5 percent increase in displaced households in North Kivu, South Kivu, Ituri and Tanganyika.

UNFPA's response reached 135,100 individuals with protection from gender-based violence (GBV) and 71,000 with sexual and reproductive health (SRH) services. During the reporting period, 150 midwives have been trained in clinical management of rape, and 145 reproductive health kits were deployed across eastern provinces of DRC to meet the needs of 123,183 women.

According to data from the GBV Area of Responsibility (AoR), in October, GBV actors in the DRC reached 349,447 individuals—with women comprising 62%—with medical care, psychosocial support, legal assistance, information and awareness activities, and other services. However, this represents achieving 40% of the 864,126 persons targeted, highlighting a significant gap in the response. Coverage varied significantly across provinces. South Kivu recorded the highest coverage at 83% of the target population, while Ituri reported the lowest at 17%, largely due to ongoing insecurity and access constraints.



5.5M

Total people affected¹



3.5M

People internally displaced²



1.2M

Women of reproductive age²



269,550

Estimated pregnant women³



933,730

People targeted w/ SRH services



459,000

People targeted w/ GBV programmes

¹ OCHA Protection Cluster. February 2025.

² [IOM. DRC – Internal Displacement Overview. October 2025.](#)

Situation Overview

The humanitarian and protection landscape in the eastern provinces of the DRC—particularly North Kivu, South Kivu, Ituri and Tanganyika—continued to deteriorate rapidly throughout October 2025. Intensified armed conflict hindered peace efforts and triggered massive population movements. In October 2025 alone, a 15.5 percent increase in displaced households was recorded compared to the previous month. This included 55,334 households displaced in North Kivu, 23,590 households in South Kivu, 13,898 households in Ituri, and 4,680 households in Tanganyika.³

Humanitarian access has continued to be restricted by insecurity, bureaucratic impediments and deteriorating road infrastructure. In affected areas of North Kivu such as Kiwanja–Ishasha, it hinders women and girls' ability to access health structures and services.

Peace efforts were restarted with talks between the DRC and Rwanda on October 21 in Washington under the mediation of the United States of America. An international conference took place on October 30 in Paris to mobilize support for peace and prosperity in the Great Lakes regions.

The number of people facing emergency levels of hunger in eastern DRC has nearly doubled in just one year. In the provinces of North Kivu, South Kivu, Ituri, and Tanganyika, one in three people is now experiencing crisis-level hunger or worse—affecting over 10 million individuals. Pregnant women are particularly vulnerable, as malnutrition poses life-threatening risks to both them and their unborn babies, increasing the likelihood of miscarriage, premature birth, and low birth weight.⁴

UNFPA Response

Sexual and reproductive health:

- 321 UNFPA-supported midwives and 504 community health workers were deployed across 74 health facilities across the North Kivu, South Kivu and Ituri provinces.
- 145 IARH kits for clinical services were delivered to 9 health facilities in 4 health zones. This will serve 102,996 women of reproductive age and 20,187 pregnant women.
- 518,400 condoms were also delivered to the above-mentioned health facilities to supply 131,834 sexually active men for at least three months.
- 50 health facilities in Ituri and North Kivu received medical equipment and supplies, including handheld digital echography machines.
- 150 midwives in the health zones of Goma, Karisimbi, Nyirangongo, and Sake received training from UNFPA, HEAL AFRICA and the National Programme of Reproductive Health (PNSR). The training strengthen their skills in clinical management of rape, intimate partner violence, and mental health and psychosocial support.
- 50 members of Community Advisory Councils and 10 Community Health Workers received training on referral pathways for SRH, including obstetric complications, GBV, and feedback mechanisms.
- Other interventions included, data management for evidence based programming, and financial support to the operationalization of health facilities.

Gender-based violence:

- 135,100 individuals were reached with comprehensive GBV services, including professional case management and psychosocial support.

³ Crisis Analysis - DR Congo: Monthly Displacement Report, October 2025.

⁴ [WFP. Hunger surges in DRC's conflict-hit Eastern provinces yet funding gaps force WFP to scale back support. 7 November 2025.](#)

- 5,539 women and girls accessed psychosocial support and service referrals at health facilities within the Nyiragongo, Kibirizi, and Karisimbi health zones.
- 442 women and 186 girls in South Kivu accessed critical protection services through established safe spaces.
- 300 women and adolescent girls at high risk of GBV participated in entrepreneurship and technical skills training through the cash and voucher assistance (CVA) programme. This initiative, implemented across North Kivu, South Kivu, and Ituri, was designed to foster the development of sustainable income-generating activities and provide the financial foundation necessary for long-term socio-economic empowerment.
- 2,279 individuals—including 876 women, 604 adolescent girls, 453 men, and 346 adolescent boys—were reached through GBV awareness campaigns. These sessions focused on GBV prevention, the utilization of referral pathways, Accountability to Affected Populations (AAP), and the prevention of sexual exploitation and abuse (PSEA)..
- 60 health providers were briefed on core concepts on sexual exploitation and abuse (SEA) and relevant complaint mechanisms in Ituri, and staff from 4 implementing partners (HEAL Africa, SOFEPADI, RADPI, MDA) were trained on AAP.

Adolescents and youth:

- Through rehabilitated spaces adapted for adolescents and young people in Goma (Goma health zone) and Kitshanga (Mwesso health zone), 327 adolescents and youth—including 165 boys and 162 girls—were reached with information on SRH, GBV prevention, mental health and psychosocial support (MHPSS), life-skills education, and youth-friendly health initiatives. Youth also participated in community-based peacebuilding and emergency preparedness efforts and 3 qualified as winners for an entrepreneurship competition.
- 55 youth organization leaders from Goma and Kitshanga completed an intensive training programme covering leadership, reproductive health, GBV prevention, and conflict transformation, alongside specialized sessions on trauma healing and stress management.

Results Snapshot



71,884

People reached with SRH services
67% female, 33% male



72

Health facilities supported



135,100

People reached with GBV prevention,
mitigation and response activities
54% female, 46% male



9

Safe spaces for women and girls
supported



403

Dignity kits were distributed to women and girls



181

People received mental health and psychosocial support



145

Reproductive health kits provided to service delivery points to meet the needs of 1,462,960 people



300

Women and adolescent girls received vocational training to support income-generating activities

Coordination Mechanisms

The GBV AoR in the DRC elected Feminine Solidarity for Peace and Development (SOFEAPDI), a

Gender-based violence: women-led and national non-governmental organization, as co-lead with UNFPA.

As part of the GBV AoR capacity building plan, Doctors of Africa (MDA) conducted an awareness-raising workshop for the NGO ENCORE. This capacity-building effort for a national organization focused on maternal health, GBV, AAP, and PSEA.

Clinical management of rape:

The UNFPA-led Clinical Management of Rape (CMR) Task Force, comprising of 38 members, held an workshop in October to strengthen coordination and harmonize approaches for survivor response in North Kivu. Key goals included aligning their action plan, adapting indicators to the local context, and mobilizing field actors to ensure a more humane, coordinated, and sustainable effort. The discussions emphasized a comprehensive approach integrating mental health, psychosocial support, HIV prevention/care, and comprehensive abortion care centered on women, as well as the need to harmonize data management tools. Deliverables included a revised action plan detailing implementation levels across coordination, service delivery, capacity building, referrals, and monitoring, and a strategy for post-rape kits. Additionally, coordination involved a second round of mobilizing SRH decision-makers to review and cost the accelerated plan for preventing maternal and neonatal deaths.

The CMR Task Force reported that, 17,512 individual post rape kits were deployed by SRH partners to 23 health zones in the North Kivu province between the 1 September and the 31 October 2025. [While gaps still remain in providing each survivor presenting within 72 hours of a rape incidence, with a post rape kit and first line psychosocial support](#), notably in high risks health zones like Goma, Karisimbi, Walikale, great advancements have been made by SRH partners through donor support to ensure uninterrupted provision of care. In the South Kivu province, UNFPA has mobilized actors to discuss a UNFPA distribution plan of post rape kits that would largely cover needs in the province.

Sexual and reproductive health:

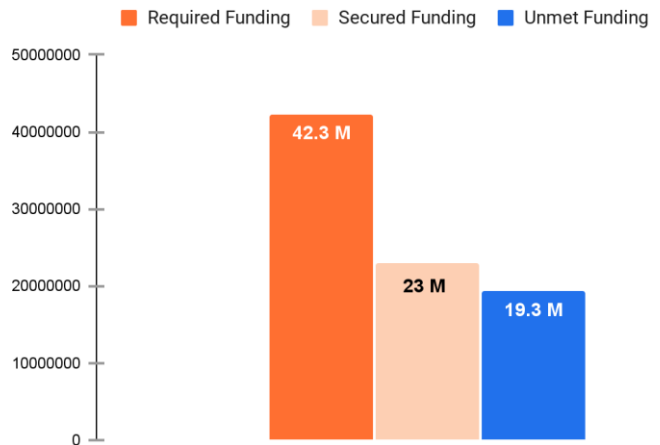
As part of Breast Cancer Awareness Month, UNFPA and SRH WG partners, AFRIYAN, COSAMED, HEAL Africa, RADPI, and the Congo Debut Initiative, in collaboration with the University of Goma, launched a breast cancer awareness and prevention campaign in Goma, under the 2025 theme "Together against breast cancer: towards an integrated national strategy for prevention, screening and care." More than 2,000 women and men from communities, particularly in churches, mosques, university campuses and humanitarian organizations, received information on breast cancer prevention, the

screening and care services available in North Kivu province. A key recommendation was to integrate awareness raising, screening and care for breast cancer patients in humanitarian response given that the protracted crisis has inadvertently affected the availability of these services.

Funding Status

UNFPA has been appealing for US \$42.3 million in 2025 to scale up and provide lifesaving SRH and GBV services to 1.8 million people across the eastern provinces of the DRC.

As of the end of October, only US \$23 million—54% of the required funding—has been secured, leaving a gap of US \$19.3 million. This shortfall directly compromises the provision of essential health and protection services, putting thousands of vulnerable women and girls in DRC at heightened risk.



Disclaimer: Funding available is based on cash received during

I spoke with midwives who continue to deliver babies under extremely difficult conditions. These frontline health workers continue to serve their communities with courage and compassion."

Shoko Arakaki, UNFPA Director of Humanitarian Response during her visit to DRC in October 2025

Current Donors

- European Civil Protection and Humanitarian Aid Operations (ECHO)
- Government of Japan
- Government of Italy
- Government of Canada
- Government of Norway
- UK Foreign, Commonwealth & Development Office (FCDO)
- UN Central Emergency Response Fund (CERF)
- UNFPA Emergency Fund / Humanitarian Thematic Fund
- Democratic Republic of Congo Humanitarian Fund (DRC HF)
- Democratic Republic of Congo / National Fund for Reparation of Victims of Sexual Violence Related to Conflict

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